Bamboo Health

June 16, 2025

Administrator Mehmet Oz, M.D., M.B.A
Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
200 Independence Avenue, S.W.
Washington, D.C. 20201

Assistant Secretary Thomas Keane, M.D., M.B.A.
Office of the Assistant Secretary for Technology Policy
and National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
330 C Street, S.W.
Washington, D.C. 20201

RE: Department of Health and Human Services, Centers for Medicare & Medicaid Services (CMS), Assistant Secretary for Technology Policy (ASTP)/ Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC) [CMS–0042–NC], RIN 0938–AV68, Request for Information; Health Technology Ecosystem

Submitted electronically via regulations.gov

Dear Administrator Oz and Assistant Secretary Keane,

On behalf of Bamboo Health (Bamboo), we appreciate the opportunity to provide feedback on the Request for Information (RFI) regarding the Health Technology Ecosystem. As purveyors and supporters of digital innovation, our comments are intended to inform future policymaking that will enhance interoperability while leveraging and building upon existing infrastructure to optimize better health outcomes for all Medicare and Medicaid beneficiaries.

With a breadth of solutions and interoperability across behavioral and physical healthcare that Bamboo Health has deep experience and established capabilities to deliver, providers across the country can now work together on a shared platform with differentiated data, insights and clinical decision support tools needed to enhance their patients' physical and behavioral health. Complete with actionable and timely clinical insights during pivotal moments, our solutions help organizations achieve optimized clinical and financial outcomes.

Bamboo has a longstanding commitment to patient care, improving public health through the deployment of technology that allows care providers to see and act upon important information in real time. We provide trusted technology solutions to federal and state governments, payers, health systems, clinicians, and pharmacies working to improve public health. At Bamboo Health, we connect payers and providers across all 50 states in one of the most interoperable care coordination networks in the nation. We also help to

advance whole person care by delivering real-time patient data and analytics pertaining to behavioral and physical health. Further, Bamboo Health is the industry leader in supporting efficient and secure real-time access to Prescription Drug Monitoring (PDMP) data and analytics directly within clinical workflows. This managed service platform provides authorized health care providers with comprehensive connectivity, patient support and engagement, and care coordination.

On behalf of Bamboo Health, we appreciate the opportunity to respond to the Health Technology Ecosystem RFI. Our responses address key barriers and opportunities across providers, payers, technology vendors, and value-based care organizations, all informed by direct experience in the field over several decades.

PROVIDERS

FHIR APIs and TEFCA [PR-5, PR-6]:
We support standards-based data exchange (e.g., SMART on FHIR). While TEFCA may at first appear duplicative of other established frameworks or networks, it should continue to exist alongside these other frameworks or networks. The level of data sensitivity or the need for compliance controls (e.g. PDMP data) supersedes TEFCA-level interoperability and the networks have demonstrated the ability to sustain and scale based on the value they provide to stakeholders. At a minimum, they should co-exist, encouraging further innovation.

Recommendations for Data Availability and Burden Reduction [PR-7]:
CMS and ASTP/ONC can reduce provider burden by:

- Implementing data timeliness standards based on the data type and use case.
- Mandating accessible, standardized data sets from payer claims data across Medicare and Medicaid programs.
- Encouraging the reuse of existing workflows and metrics to support interoperability without increasing provider burden.
- Supporting technology and access that identifies gaps in care that can be addressed with care navigation services.

Bulk FHIR for Quality Reporting [PR-8]:
Bulk FHIR exports should be leveraged to enable payers – including CMS – to calculate quality metrics, reducing provider burden. CMS should also establish registry requirements that support real-time quality data access for clinicians.

PAYERS

API Acceleration [PA-2]:
We urge CMS to mandate API implementation across payers to unlock data flow that improves care and reduces costs. This should mirror efforts like Blue Button 2.0 and Data at the Point of Care.

Standardization of Provider Submissions [PA-5]:
Payers can support providers by adopting a unified data submission specification, eliminating the need for unique custom formats. Automation between provider and payer systems should be a key priority.

TECHNOLOGY VENDORS & NETWORKS

Driving Developer Engagement [TD-1, TD-2]:
CMS and ASTP/ONC should consider the following policy levers: funding technology development targeting high-risk and underserved populations; mandating integration pathways into core clinical systems; establishing standards for patient-generated data ingestion; and rethinking time-based billing to support asynchronous, digital care modalities.

Digital Identity and APIs [TD-3, TD-4]:
We support OpenID Connect and other secure digital identity protocols. While open APIs are ideal, proprietary APIs may be necessary for near-term progress to sustain momentum while open APIs are being built and adopted

Ensuring Appropriate Management of Sensitive Health Data [TD-12 through TD-17]:
Interoperability frameworks must recognize that uniform access or control standards will not be appropriate for all types of health information. For example, Prescription Drug Monitoring Program (PDMP) data involves heightened sensitivity and is governed by complex, state-specific regulations that necessitate stricter user access controls and use case limitations. Indeed, including further similar data sets such as methadone within PDMPs would help to improve clinical decision support. A standardized, one-size-fits-all approach could conflict with these carefully crafted safeguards. CMS and ASTP/ONC must recognize these distinctions in policy development. At the same time, we support the inclusion of complementary, less sensitive data—when aligned with PDMP objectives—into interoperability efforts, if integration is managed through strong governance and technical safeguards.

Fostering Commercially Viable Interoperability Networks [TD-17]:
A variety of commercially viable network models have successfully integrated interoperability as a foundational element in their design. These networks have demonstrated the ability to sustain, and scale based on the value they provide to stakeholders, **independent of ongoing public funding**. As CMS and ASTP/ONC evaluate future interoperability strategies, they should assess whether the market has matured to a point where long-term self-sustainability can be expected as a baseline, allowing federal efforts to focus on enabling equitable access, innovation, and safeguards rather than subsidizing core operations.

VALUE-BASED CARE ORGANIZATIONS

Adoption Barriers in ACOs [VB-1]:
Key challenges include distrust of Artificial Intelligence (AI), lack of system integration, and billing models that disincentivize digital care. Overcoming these requires both cultural and regulatory shifts. CMS could better incentivize the adoption of digital care solutions by reimbursing asynchronous modalities.

Integrated Care Navigation [VB-2]:
Integrating and incentivizing “real-time” coordinated care for beneficiaries, including dual eligibles, within the Medicare Fee-For-Service and Medicare Advantage programs will streamline the continuum of care, reduce redundancies, and improve overall health outcomes. Critical components of effective coordinated care for behavioral health patients include:

- Leveraging technology during pivotal care moments in the acute, transitional and outpatient settings to prompt navigation and the next best actions.
- Including standardized assessments (e.g. PHQ9, GAD7, AUDIT) to identify risks and triage patients into appropriate levels and types of care and follow disease progression over time.
- Leveraging AI and intelligent automation (iA) to drive repeatable, non-clinical task-oriented processes; summarize personal and population level data for consumption by the care team for process improvement and efficient clinical activities; and enable efficiencies in the process via "human in the loop" automation, tasking and pre-processing.
- Leveraging a trained "top of license" care team that guides behavioral health patients including scheduling, referrals, and supporting adherence to treatment plans.
- Leveraging EHR connectivity and interoperability to ensure data sharing and real-time alerts in native provider and care team workflows.
- Establishing seamless partnerships between behavioral health specialists/facilities and primary care providers
- Engaging patients and helping them navigate to optimize better outcomes, including working closely with patients to schedule and attend appointments, ensuring follow-up, and educating them on available services and post-care follow-ups and monitoring during transitions of care.
- Addressing capacity concerns by incorporating and building in-person and virtual care options.

IT and Data Essentials [VB-3, VB-4]:
Success in value-based care hinges on interoperable systems that reduce administrative burden; enable access to comprehensive data types including clinical, claims, and patient-generated data; and offer equal technology standards for APM and non-APM organizations to ensure consistent care quality.

We thank CMS and ASTP/ONC for their leadership and renewed focus on interoperability and digital innovation. We welcome continued dialogue and are happy to provide additional input. Don't hesitate to contact Vatsala Kapur, Senior Vice President, at vkapur@bamboohealth.com.

Sincerely,

Jeff Smith
Chief Executive Officer, Bamboo Health